

Dyadic Digital Health: Consent, Relationship Boundaries, and AI Separation

Dyadic Digital Health: Consent, Relationship Boundaries, and AI Separation I

Dyadic Digital Health: Consent, Relationship Boundaries, and AI Separation II

Dyadic digital health research is important because DigiPPPiP addresses two interdependent people rather than an isolated user. Scoping work on dyadic digital health interventions shows that digital systems can be designed around partner interdependence, shared goals, and reciprocal support [shaffer2022dyadic]. Dyadic health science also sets a stronger methodological bar: high-quality work should include both dyad members, focus on a meaningful relationship, and ground the dyadic rationale in theory rather than convenience sampling [wilson2024dyadichealth]. Recent dyadic digital-health design work shows how shared-care modules can expose live reports, care roles, messaging, timelines, and resources for patient-caregiver pairs [benmessaoud2023dyadicmodule]. Digital interventions for relationship quality show why low-barrier formats matter: cost, stigma, geography, and scheduling can prevent couples from accessing formal services [kernova2025relationship]. Qualitative work on remote Dyadic Developmental Psychotherapy sharpens the remote-care boundary: online delivery